

1. Administrative & Study Identification

Full Study Title: A Study of BMS-986482 Alone or as Combination Therapy in Participants With Advanced Solid Tumors **Short Title/ Acronym:** CA236-0001 / BMS-986482 in Advanced Solid Tumors **Protocol Number:** CA236-0001 **NCT Number:** NCT06697197 **Sponsor/ Company Name:** Bristol Myers Squibb **Investigational Product:** BMS-986482 (an immuno-oncology optimized CELMoD™ pan-degrader of IKZF1-4) **Phase of Development:** Phase I/II **Trial Status:** Recruiting (Currently recruiting participants) **Medical Monitor:** Omid Hamid / Clinical Operations Lead, Bristol Myers Squibb

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To test the safety, tolerability, and efficacy of BMS-986482 administered alone and as combination therapy, and to determine if it can slow or stop the growth of advanced solid tumors and improve survival.

Secondary Objectives: To establish the maximum tolerated dose (MTD) and/or Recommended Phase 2 Dose (RP2D), characterize the pharmacokinetic (PK) profile, and assess preliminary anti-tumor activity (ORR, PFS, OS).

2.2 Background & Rationale Regulatory T (Treg) cell suppression and effector T (Teff) cell dysfunction are major drivers of immunotherapy non-response and relapse in cancer patients. The shared roles of the Ikaros zinc finger family (IKZF) of transcription factors in regulating antitumor immune responses is crucial. BMS-986482 is a novel CELMoD™ that acts as a pan-degrader of IKZF1-4, demonstrating rapid, selective, and CRBN-dependent degradation of these transcription factors. Preclinical models show that pan-degradation of IKZF1-4 with BMS-986482 provides robust tumor growth inhibition. This study tests this mechanism in humans to reverse immunosuppressive programs and provide clinical benefit for advanced solid malignancies.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Monotherapy) and Active Combination Arms **Blinding:** Open-label **Randomization:** Assigned to specific experimental treatment cohorts (e.g., Part 1A, 1B1, 1B2, 1B3, 1C, 2A, 2B1, etc.)

3.2 Planned Enrollment & Duration Target \$N\$: 413 evaluable patients **Number of Sites:** Global, Multicenter **Estimated Study Start:** 17-Feb-2025 **Estimated Primary Completion:** 04-Jan-2029

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Age ≥ 18 years or the legal age of consent in the jurisdiction where the study takes place, inclusive, at the time of signing the ICF. 2. Histologically or cytologically confirmed, advanced, unresectable/metastatic, solid malignancy measurable by RECIST v1.1. 3. Participants must have received, be refractory to, ineligible for, intolerant of, or refused existing therapy(ies) known to provide clinical benefit for the condition of the participant.

4.2 Exclusion Criteria 1. History of life-threatening immune-mediated toxicity related to prior T-cell agonist or checkpoint inhibitor therapy, except those that are unlikely to re-occur with standard countermeasures. 2. Any significant acute or chronic medical illness which would interfere with study intervention or follow-up in the opinion of the investigator. 3. Other protocol-defined Inclusion/Exclusion criteria apply.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Administered in dose escalation (Part 1) and dose expansion (Part 2) cohorts. **Route of Administration:** BMS-986482 administered with or without Intravenous combinations. **Combinations Evaluated:** Nivolumab, rHuPH20, relatlimab, and bevacizumab (Trade Names: Avastin, Vegzelma, Alymsys, Zirabev, Mvasi; ATC Codes: S01LA08, L01FG01) depending on the assigned study arm. **Duration of Treatment:** Until disease progression, unacceptable toxicity, or withdrawal of consent.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive care for solid tumors. **Prohibited:** Concurrent investigational therapies and active immunosuppressive medications not otherwise specified in the protocol.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent, Medical History, Tumor Imaging (RECIST v1.1), Lab Assessments
Baseline	Day 1	First Dose of BMS-986482 $\pm$ Combination, PK/PD Baseline Sampling	Interim Cycle-dependent Safety Labs, Efficacy Assessment (Tumor scans every 8-12 weeks), DLT evaluations
End of Study	At Progression	Final Vital	

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: Number of participants with Adverse Events (AEs), Serious Adverse Events (SAEs), and AEs meeting protocol-defined Dose-Limiting Toxicity (DLT) criteria to evaluate the safety and tolerability of BMS-986482 alone and in combination. **Measurement Method:** Clinical assessments and laboratory evaluations graded per National Cancer Institute - Common Terminology Criteria for Adverse Events version 5.0 (NCI-CTCAE v5.0).

7.2 Secondary & Exploratory Endpoints Objective Response Rate (ORR) measured by RECIST v1.1. Progression-Free Survival (PFS) and Overall Survival (OS). Pharmacokinetic parameters: Concentration at the end of infusion (C_{max}), Time of maximum observed concentration (T_{max}), and Area under the concentration-time curve in one dosing interval (AUC(TAU)).

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Continuous monitoring via HCP assessment, laboratory exams, and physical findings throughout the treatment period. Focus on immune-mediated toxicities. **Serious Adverse Events (SAEs):** Standard reporting timelines strictly enforced (typically 24 hours). **Data Monitoring Committee (DMC):** Internal Safety Review Committee (SRC) handles dose-escalation clearance and cohort expansions.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients receiving ≥ 1 dose of study drug); Efficacy Set / Intent-to-Treat (ITT) population for response evaluation. **Sample Size Justification:** $N = 413$; calculated to safely estimate the MTD in Part 1 and provide adequate statistical power for early efficacy signals (ORR) in Part 2 expansion cohorts. **Handling of Missing Data:** Standard survival analysis methods (e.g., Kaplan-Meier with right-censoring) for PFS and OS.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki. **Monitoring Plan:** Regular onsite and remote monitoring visits to evaluate protocol adherence, safety, and RECIST tracking. **Audit**

Trail: Data entered into validated eCRFs, locked post-clarification.

11. Study Identifiers & Governance

Primary ID: CA236-0001 **Posting ID:** 14719934922846212 **Registry Number:** NCT06697197 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study of BMS-986482 Alone or as Combination Therapy in Participants With Advanced Solid Tumors **Official Regulatory Title:** A Phase 1/2 Study of BMS-986482 as Monotherapy or Combination Therapy in Participants With Advanced Solid Tumors

12. Clinical Framework (Oncology Specific)

Disease Areas / Primary Condition: Advanced Solid Tumors / Advanced Solid Malignancies **Preferred UMLS Name:** Advanced Malignant Solid Neoplasm **Semantic Type:** Neoplastic Process **Definition:** A malignant solid neoplasm that has spread extensively to other anatomic sites or is no longer responding to treatment. **SNOMED ID:** 39958004 **SNOMED Hierarchy:** 785317006 | Advanced solid malignant neoplasm | Advanced solid malignant neoplasm (disorder) **Diagnosis Threshold:** Histologically or cytologically confirmed, unresectable/metastatic **Medical Methodology:** CELMoD™ IKZF1-4 pan-degrader / Immuno-oncology **Optimization Target:** Maximum Tolerated Dose (MTD) / Recommended Phase 2 Dose (RP2D) and immune program reversal

13. Study Procedures & Methodology

Management Pathway: Dose Escalation (Part 1) mapping into Dose Expansion (Part 2) cohorts across combinations **Education Protocol (HCP):** Site initiation visits detailing DLT definitions and immune-mediated adverse event management. **Education Protocol (Patient):** Informed consent detailing mechanisms of action, PK tracking, and safety reporting. **Quality Audit Mechanism:** Source Data Verification (SDV) and ongoing Clinical Research Associate (CRA) reviews.

14. Observation & Follow-Up Schedule

Total Duration: Up to approx. 48 months (until primary completion in Jan 2029) **Onsite Visit Cadence:** Frequently during Cycle 1 for PK/PD and DLT monitoring, transitioning to Q2W/Q4W per specific arm protocol. **Remote Monitoring Frequency:** Between cycles / for survival follow-up post-progression. **Checklist Review Interval:** Tumor imaging (RECIST v1.1) roughly every 8 weeks.

15. Sign-off & Approval

Role	Name	Signature	Date		:---	:---	:---	:---
Principal Investigator			Omid Hamid	_____	[DD-MMM-YYYY]			
Clinical Operations Lead			BMS Clinical Rep	_____	[DD-MMM-YYYY]			
	Lead Statistician	BMS Biostatistics	_____		[DD-MMM-YYYY]			